

Partograph

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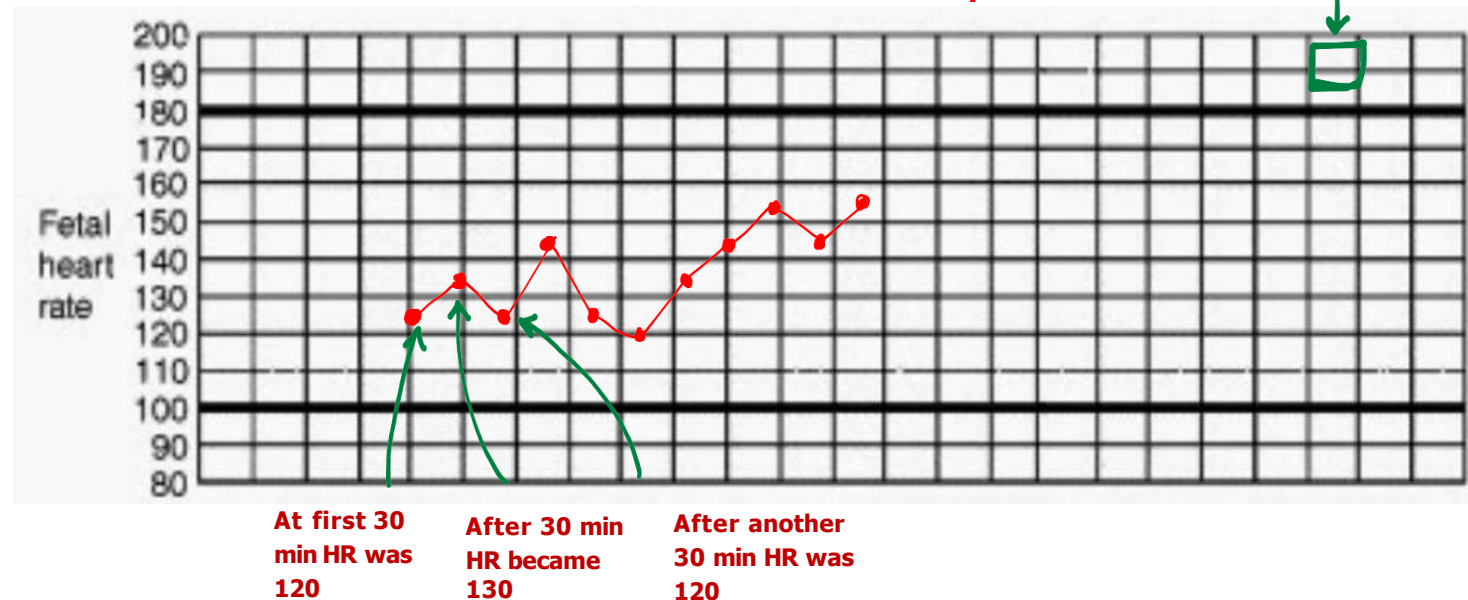
- The partograph is a graphical presentation of the progress of labour, and of fetal and maternal condition during labour. It is the best tool to help you detect whether labour is progressing normally or abnormally, and to warn you as soon as possible if there are signs of fetal distress or if the mother's vital signs deviate from the normal range.
- You need to monitor a labouring mother carefully. Remember that a labour that is progressing well requires your help less than a labour that is progressing abnormally. Documenting your findings on the partograph during the labour enables you to know quickly if something is going wrong, and whether you should refer the mother to the nearest higher health facility or hospital for further evaluation and intervention.
- Write the name and age of the mother, her 'gravida' (number of pregnancy regardless the outcomes, the current pregnancy should be included) and 'para' (number of births after 24. Weeks of gestation including alive or dead fetus) status, her health facility or hospital registration number, the date and time when you first attended her for the delivery, and the time the fetal membranes ruptured (her 'waters broke').

Name	Gravida	Para	Hospital Number
Date of admission	Time of admission	Ruptured membranes	hours

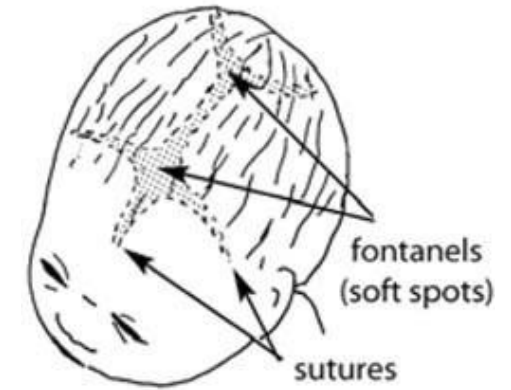
Fetal heart rate

- The fetal heart rate is recorded at the top of the partograph every 30 min in the first stage of labour and every 5 minutes in the second stage.
- Count every five minutes if the amniotic fluid (called liquor on the partograph) contains thick green or black meconium.
- Whenever the fetal membranes rupture, you should check the heart rate because occasionally there may be cord prolapse and compression, or placental abruption as the amniotic fluid gushes out.
- The normal fetal heart rate at term (37 weeks and more) is in the range of 110–160 beats/minute

Each square for the FHR on the partograph represents 30 minutes.



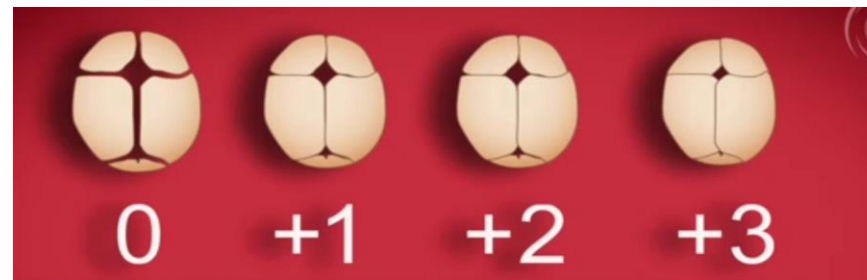
Moulding



- The five separate bones of the fetal skull are joined together by sutures, which are quite flexible during the birth, and there are also two larger soft areas called fontanelles. Movement in the sutures and fontanelles allows the skull bones to overlap each other to some extent as the head is forced down the birth canal by the contractions of the uterus. The extent of overlapping of fetal skull bones is called moulding, and it can produce a pointed or flattened shape to the baby's head when it is born
- Whenever you detect moulding in the fetal skull as the baby is moving down the birth canal, you have to be more careful in evaluating the mother for possible disproportion between her pelvic opening and the size of the baby's head.
- Make sure that the pelvic opening is large enough for the baby to pass through.
- A small pelvis is a frequent cause of prolonged and obstructed labour

- To identify moulding

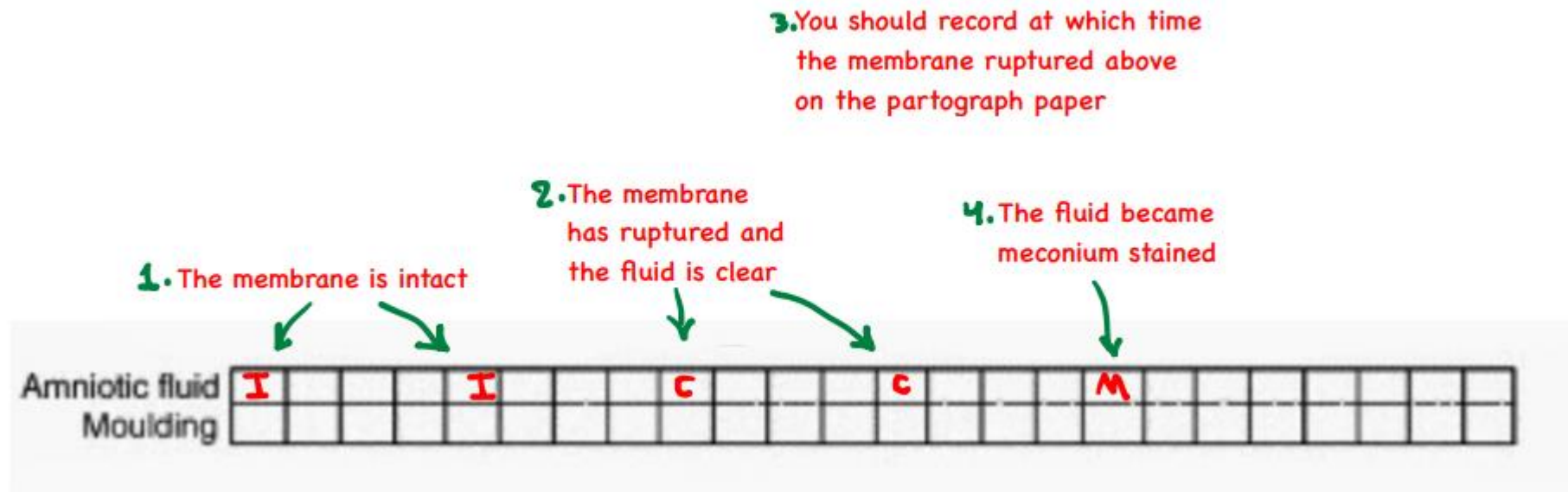
- First palpate the suture lines on the fetal head and appreciate whether the following conditions apply. The skull bones that are most likely to overlap are the parietal bones, which are joined by the sagittal suture, and have the anterior and posterior fontanelles to the front and back.
- -If the bones are separated and the sutures can be felt easily, degree 0.
- -If sutures apposed: This is when adjacent skull bones are touching each other, but are not overlapping. This is called degree 1 moulding (+1).
- -If sutures overlapped but reducible: This is when you feel that one skull bone is overlapping another, but when you gently push the overlapped bone it goes back easily. This is called degree 2 moulding (+2).
- - If sutures overlapped and not reducible: This is when you feel that one skull bone is overlapping another, but when you try to push the overlapped bone, it does not go back. This is called degree 3 moulding (+3) the baby should be delivered at this degree.
- If you find +3 moulding with poor progress of labour (there is no or small head descent), this may indicate that the labour is at increased risk of becoming obstructed.



Amniotic fluid																							
Moulding	0	0	0	0	+1	+1	+1	+1	+2	+2	+2	+2	+3	+3	+3	+3							

Amniotic fluid

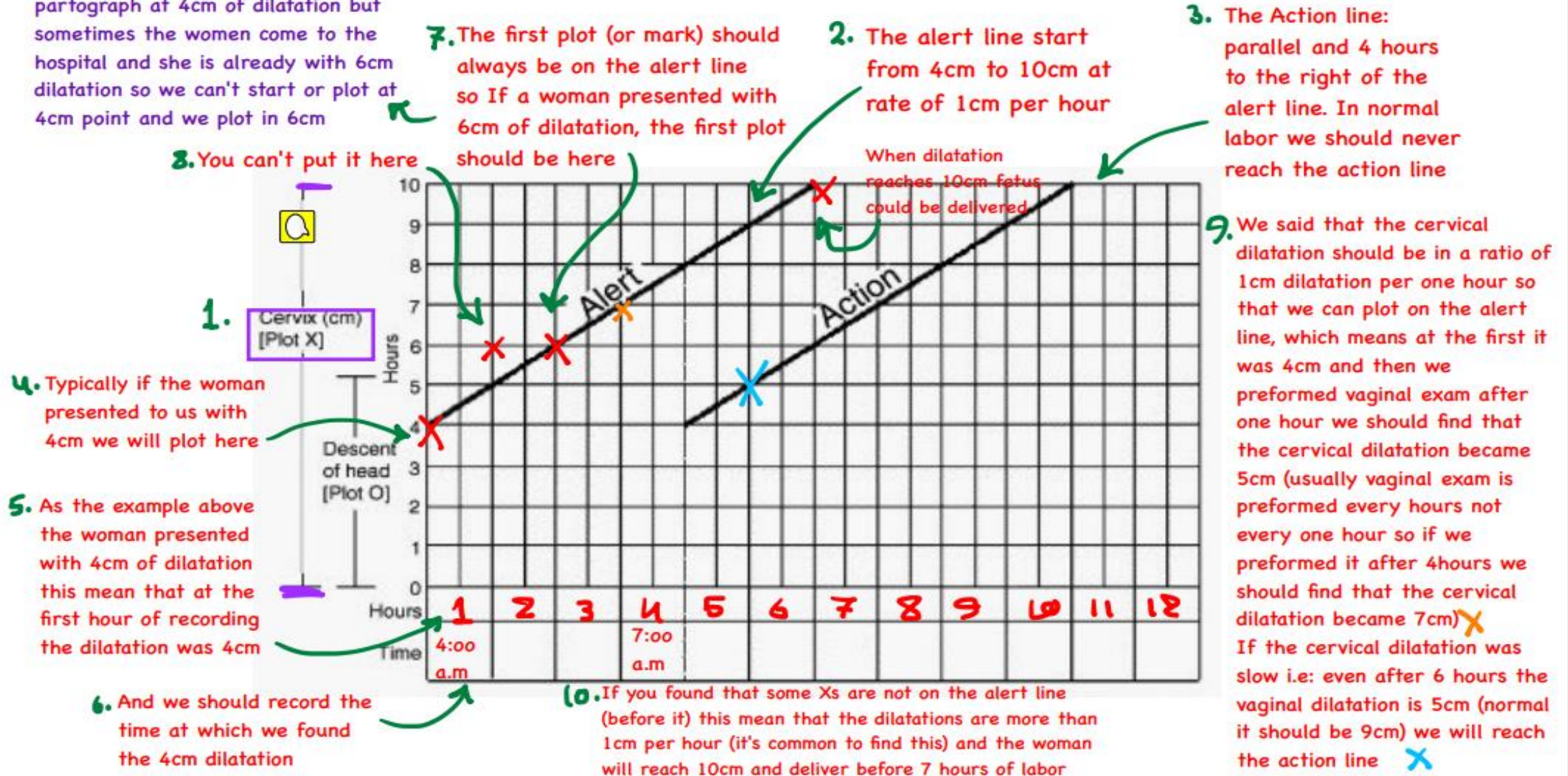
- Record the state of amniotic at each vaginal examination
- If the membrane is still intact mark **(I)** if the membrane is ruptured you can mark with **(R)**
- If the membrane is ruptured you should notice the color of the amniotic fluid, if it was clear mark **(C)** if it was meconium stained mark **(M)**, if it was blood stained mark **(B)** and if the amniotic fluid is absent mark **(A)**.



Recording cervical dilatation and fetal decent :

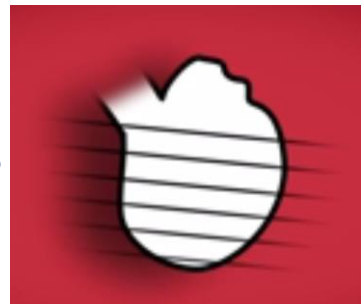
- We start recording partograph only when the cervical dilatation reaches 4cm !!
- Cervical dilatation is assessed every vaginal examination (every four hours) and marked with X

Although we said that we should start partograph at 4cm of dilatation but sometimes the women come to the hospital and she is already with 6cm dilatation so we can't start or plot at 4cm point and we plot in 6cm



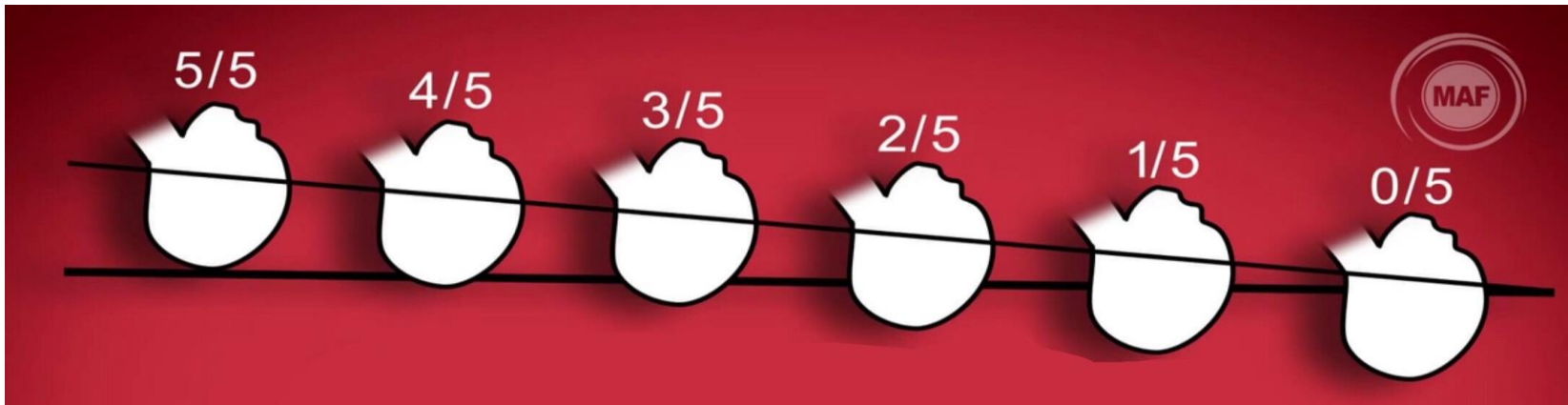
- Fetal descent

- We assess the descent of the head by #abdominal palpation
- we divide the head into five fifths, and feel how many fifths are palpable above the pubic symphysis



- It is recorded as a small O at the same time as every vaginal exam. (As we record the cervical dilatation we should preform abdominal palpation to record the fetal descent at this stage of dilatation)

- if you palpate 5 fifths the mean that the fetal head is not engaged or descent (5)
- if you didn't find any fifth to palpate this means that the fetus has fully descended or engaged (0)

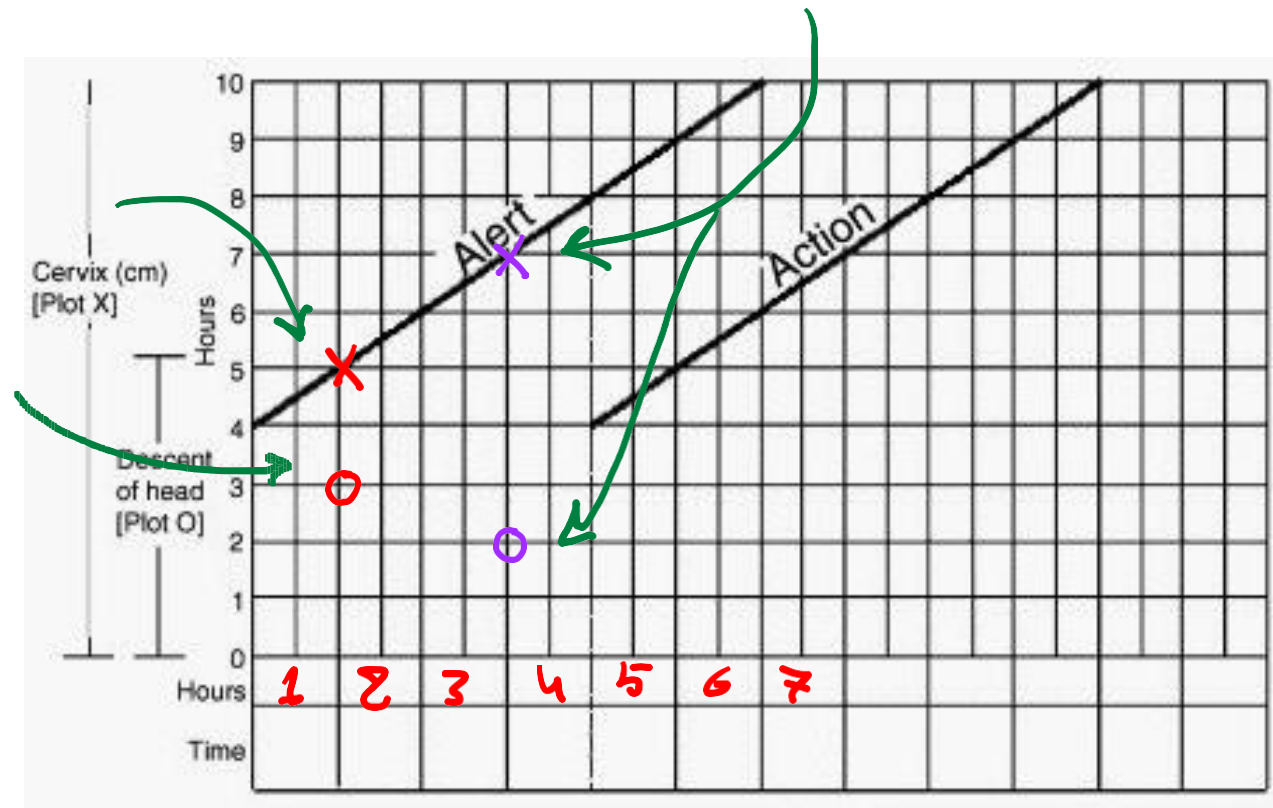


- We record the cervical dilatation and the descent at the same time because they are related to each other (if there is speculation dilatation there should be descent that is proportional to this dilatation) but if we found that there is adequate dilatation but the head is not fully descended this mean that either the baby is too large or the pelvis is too small which both prevent descent and engagement of the head

- - Example: woman presented with 5cm cervical dilatation we preformed abdominal palpation and we found that 3 fifths of the fetal head are palpable after 2 hours the vaginal exam. Revealed 7cm dilatation and abdominal palpation we found that 2 fifths are palpable

After 2 hours the dilatation was 7cm and descent was 2

Now in order to record we said that we should record the descent at the same time of the dilatation so we should plot the 5cm dilatation as X on the alert line then at the same axis we plot the descent as an O



If the woman presented with 5cm dilatation or more we plot X on the alert line parallel to the 5cm.



All other observations will be plotted at the same axis or after the axis that is indicated by the centimeters of dilatation on the alert line at the time of admission .



Name	Gravida	Para	Hospital Number
Date of admission	Time of admission	Ruptured membranes	hours

200
190
180
170
160
150
140
130
120
110
100
90
80

Fetal heart rate

Amniotic fluid Moulding

10
9
8
7
6
5
4
3
2
1
0

Cervix (cm)
[Plot X]

Descent of head
[Plot O]

Hours

Time

5
4
3
2
1

Contractions per 10 mins

Oxytocin U/L
drops/min

Drugs given and IV fluids

180
170
160
150
140
130
120
110
100
90
80
70
60

Pulse
and
BP

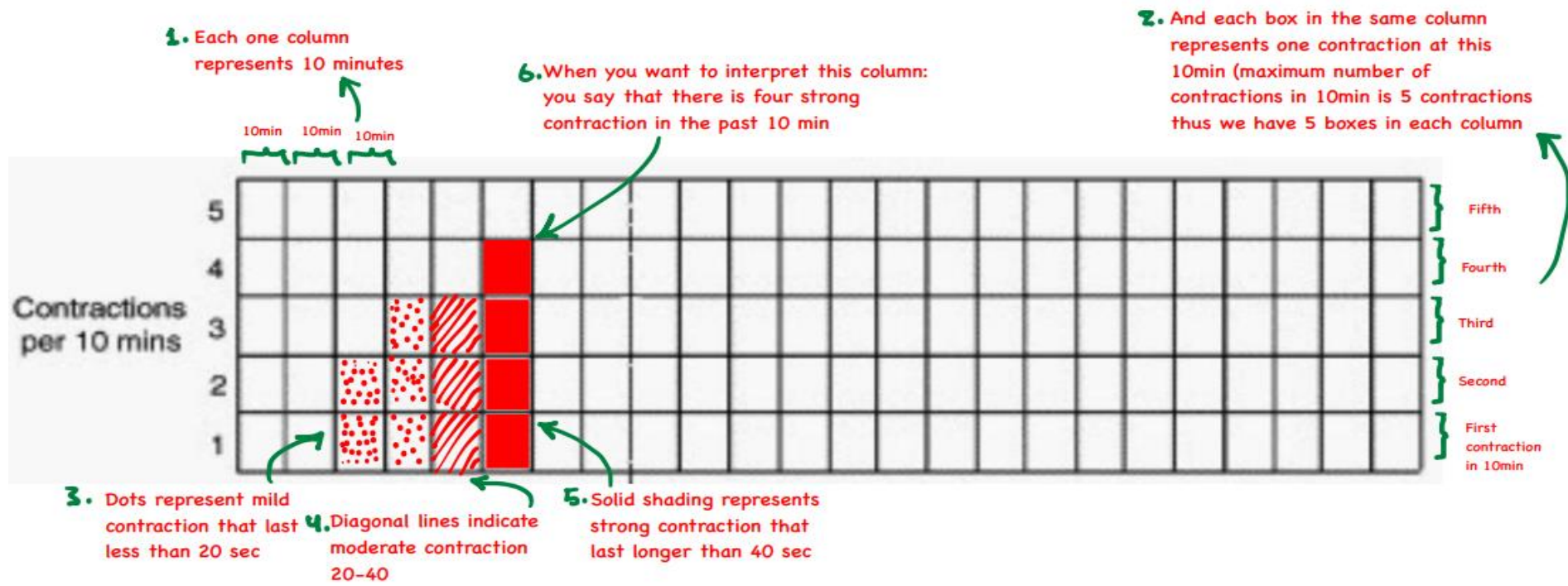
Temp °C

Urine

protein
acetone
volume

- Uterine contractions

- Good uterine contractions are necessary for good progress of labour.
- Normally, contractions become more frequent and last longer as labour progresses.
- Contractions are recorded over 10 minutes every 30 minutes on the partograph in their own section.
- At the left hand side is written 'Contractions per 10 mins' and the scale is numbered from 1–5.
- Each square represents one contraction, so that if two contractions are felt in 10 minutes, you should shade two squares.
- The way you shade the square, will indicate the duration of each contraction by using the symbols shown in the Figure below.



- If there was administration of Oxytocin during labour you should record the amount of oxytocin and the amount of iv fluid that it has been added to

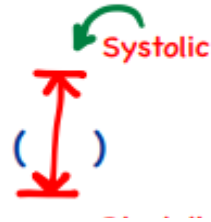
[illegible]

- Also record drugs and iv fluid that has been given to the patient during labor

Drugs given and IV fluids			metronidazole								
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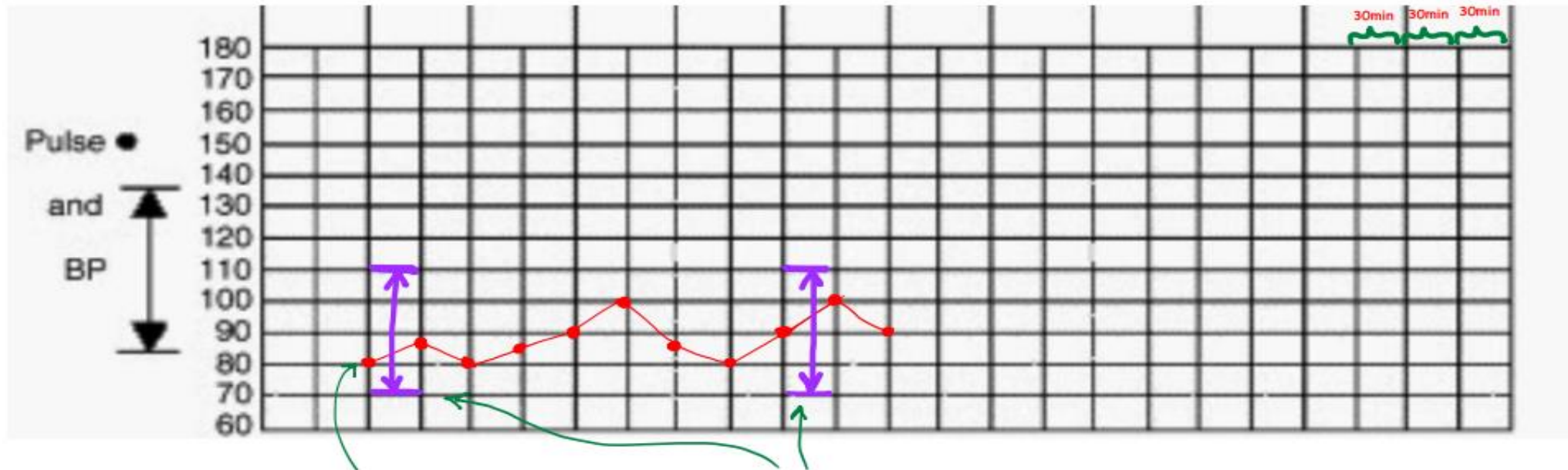
Maternal vital signs:

- Blood pressure is measured every four hours, marked with ()
- Pulse is recorded every 30 minutes marked with (●)



1. Each column represents 30 minutes

30min 30min 30min



2. The pulse was 80, after 30 min it became 90, and after another 30min it returned to 80,

3. The BP was 110/70 and after four hours it was still 110/70

Thank you